

The Global Burden of Mental Illness: Prevalence, Impact, and the Treatment Gap

A data-driven overview of global mental health conditions,
their disease burden, and the persistent gap between need and treatment.

Global Data · 2023 · Public Health Policy Analysts & Program Planners

Source: Our World in Data / IHME, Global Burden of Disease (2025)
Dattani, Rodés-Guirao, Ritchie & Roser (2023)

Hundreds of Millions Affected Worldwide

The scale of mental illness demands policy attention

1 in 3

Women

will experience major
depression in their lifetime

1 in 5

Men

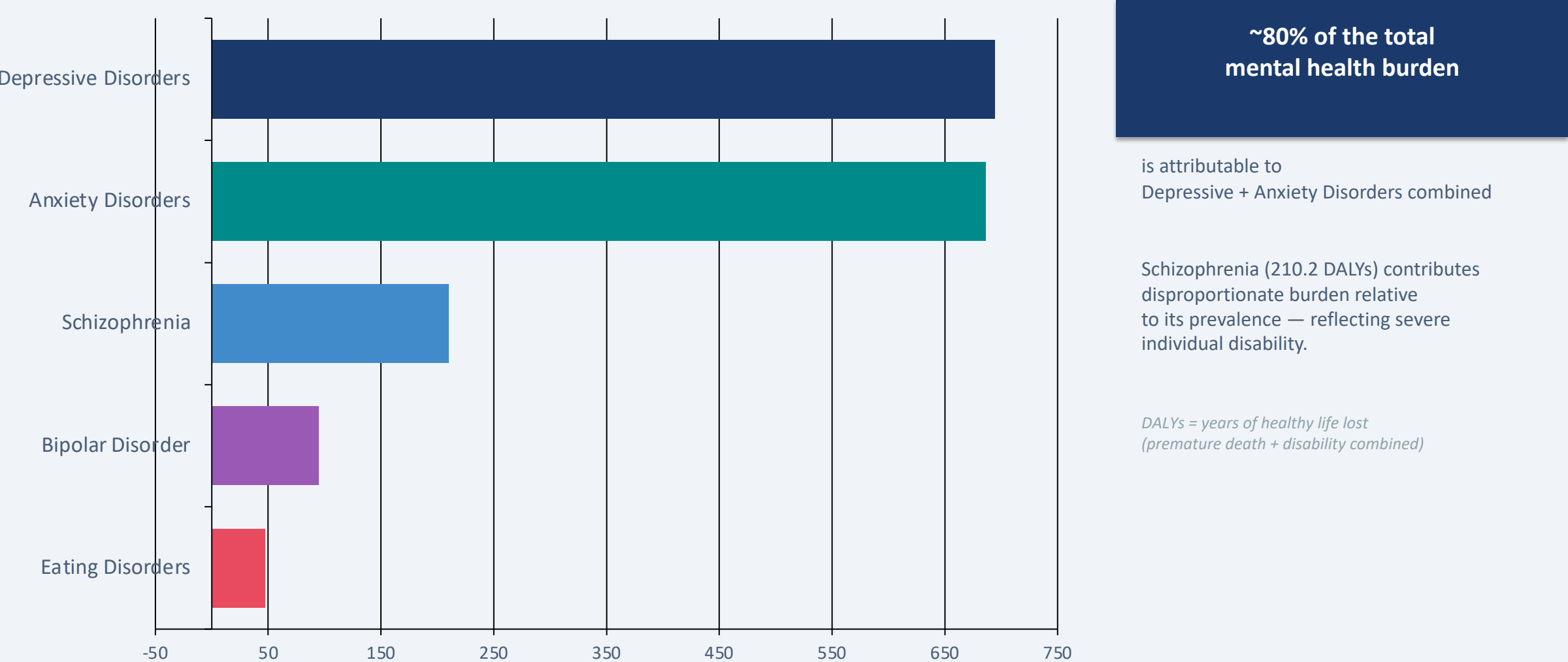
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Beyond Depression

- **Anxiety disorders:** affect hundreds of millions globally each year
- **Schizophrenia & Bipolar:** less prevalent but cause profound disability
- **Eating disorders:** contribute meaningful burden, especially in younger populations
- **All conditions:** are treatable — yet treatment often falls short

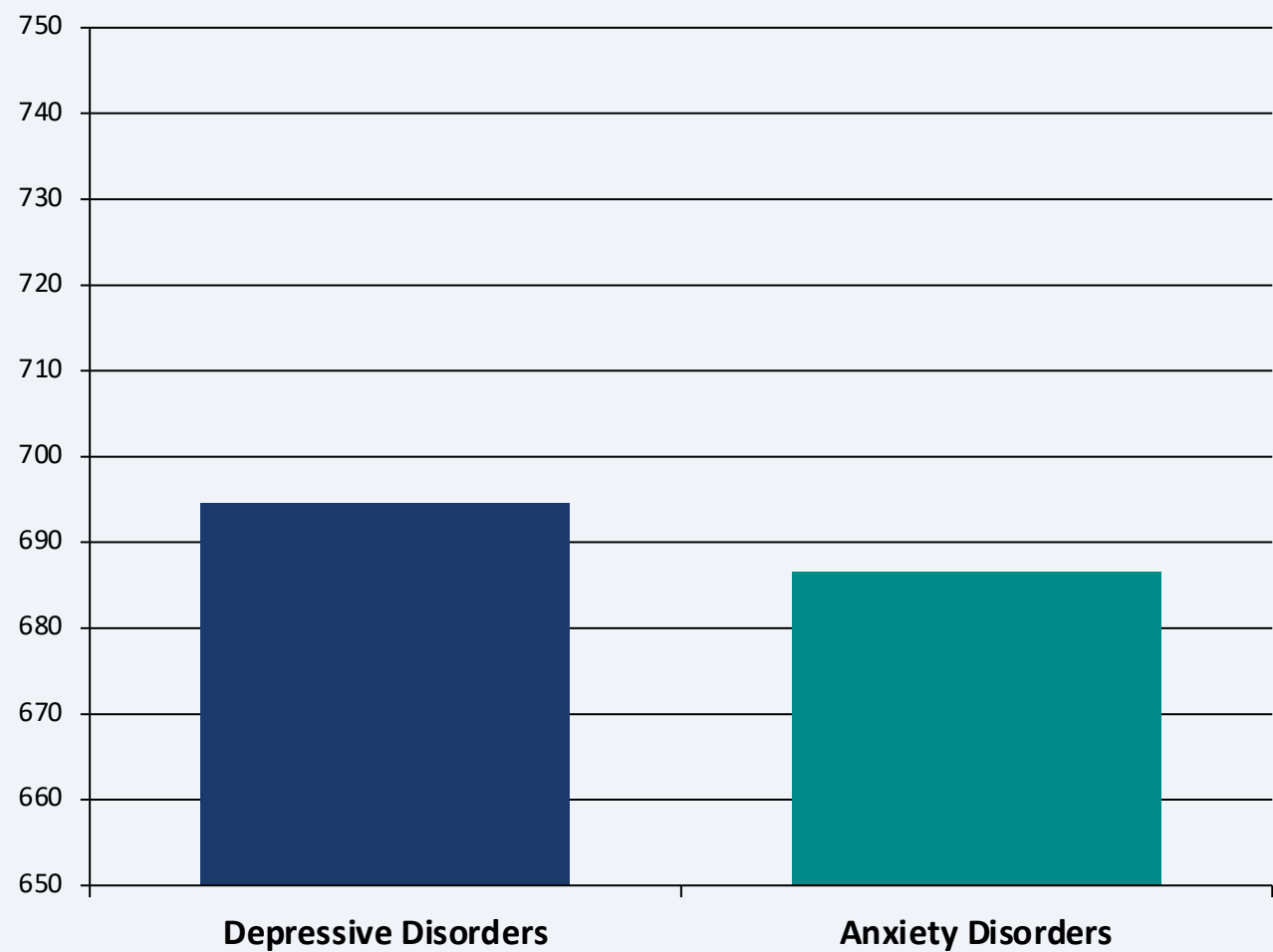
Depression & Anxiety Dominate the Global Mental Health Burden

DALYs per 100,000 people — World, 2023 (disability-adjusted life years)



Depression and Anxiety Carry Nearly Equal Disease Burden

Side-by-side comparison — DALYs per 100,000, World 2023



694.6 vs 686.5

DALYs per 100,000 — difference of only 8.1

Near-identical burden — different populations

Depression and anxiety together account for roughly 80% of the five-category mental health DALY total.

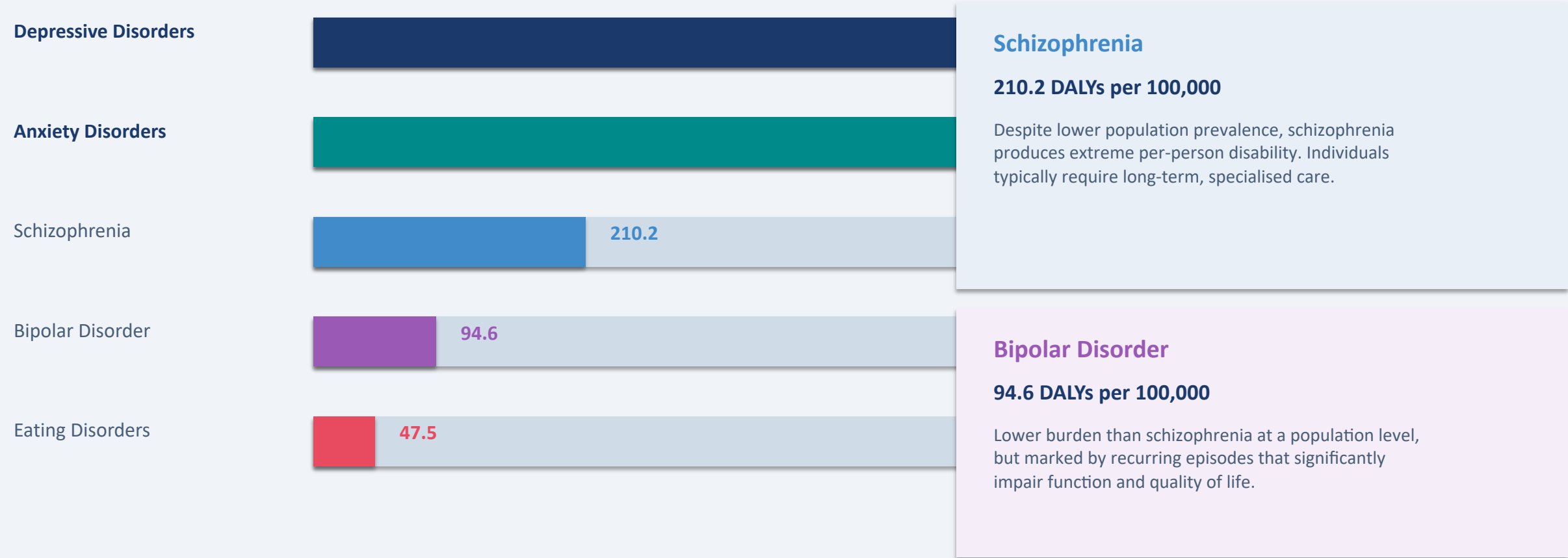
This parity has direct policy implications:

- Anxiety disorders are as costly as depression in years of healthy life lost
- Integrated treatment pathways can serve both populations
- Screening programmes should address both conditions simultaneously

Combined DALYs (depression + anxiety): ~1,381 per 100,000

Schizophrenia & Bipolar: Lower Prevalence, Severe Individual Impact

Contrasting population-level burden with individual disability



Policy implication: While scaling common-disorder interventions is the top priority, specialised services for severe conditions must be preserved.

Treatment Is Available — But Often Lacking or Poor Quality

The persistent gap between need and care

Stigma

Many people are uncomfortable sharing symptoms with healthcare providers or even family, suppressing both help-seeking and accurate prevalence data.

Access

Treatment is especially lacking in low- and middle-income countries, where mental health infrastructure and trained providers are insufficient.

Quality

Even where services exist, the quality of care may be inadequate — poorly trained staff, outdated protocols, or insufficient follow-up.

Underreporting

Discomfort with disclosure makes it hard to estimate true prevalence. Actual burden is likely higher than reported figures suggest.

Mental illnesses ARE treatable — the gap is one of access, stigma, and investment, not of therapeutic possibility.

How People Cope: Medication, Faith, and Family Conversations

Global survey data reveals wide variation in coping strategies



Prescribed Medication

Common in higher-income settings where healthcare access is available. Pharmacological treatment remains a cornerstone of clinical management for depression, anxiety, and severe disorders.



Religious / Spiritual Activities

Prevalent in many regions worldwide, particularly in lower-income settings. Spiritual practices provide community, meaning, and emotional regulation outside formal health systems.



Talking to Friends or Family

A widely reported coping mechanism across all income levels. Informal social support networks serve as a first-line response and can facilitate pathways to professional care.

Policy responses must be culturally sensitive — a one-size-fits-all approach will fail given the diversity of help-seeking behaviours across countries and cultures.

Key Takeaways & Policy Implications

1

Depression & Anxiety dominate

Each generating ~690 DALYs per 100,000, they account for roughly 80% of the five-category mental health burden. Integrated, scalable interventions are the highest-yield investment.

2

Schizophrenia & Bipolar carry disproportionate individual burden

With 210.2 and 94.6 DALYs per 100,000 respectively, these disorders cause severe disability per affected person. Specialised services must be maintained even as common-disorder scale-up proceeds.

3

Treatment exists — but the gap is wide

Effective therapies and medications are available, yet access, quality, and uptake remain inadequate, particularly in low- and middle-income countries.

4

Stigma suppresses both help-seeking and accurate data

Underreporting driven by discomfort with disclosure means true prevalence is likely higher than figures suggest. Anti-stigma campaigns are a precondition for closing the treatment gap.

5

Cultural sensitivity is non-negotiable

Coping strategies range from medication to spiritual practice to family support. Effective policy must accommodate this diversity rather than imposing a single model of care.